

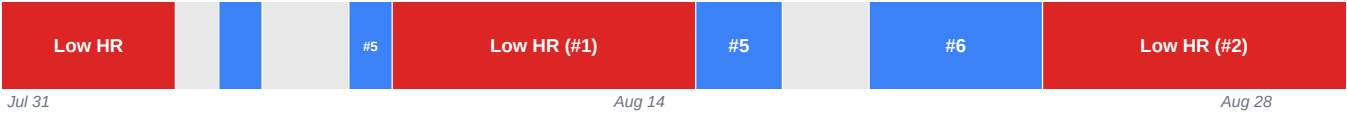
CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: TMiller **Period:** July 31 to August 30, 2024 **Report Date:** August 31, 2024 **Coverage:** 510/744h (68.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Jul 31 to Aug 30 ■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
42	78h	Low Heart Rate	1	69%

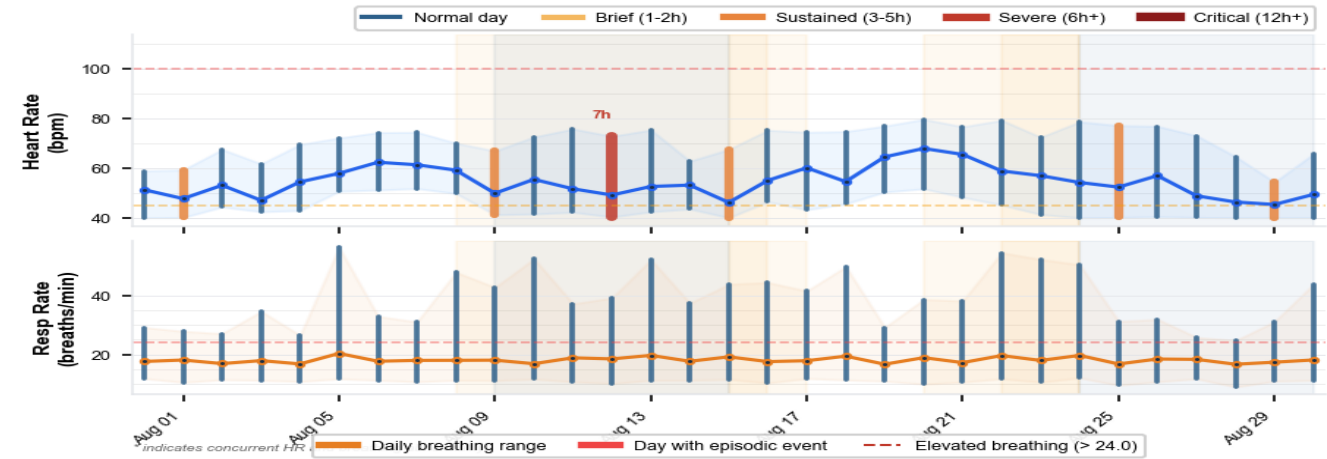
Major Findings: Sustained low HR (avg 51, min 40)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/day	Average	Date
1	Low Heart Rate	40 bpm	24h	7h	1	51 bpm	Aug 09 to Aug 15
2	Low Heart Rate	40 bpm	28h	5h	2	50 bpm	Aug 24 to Aug 30
3	High Breathing	45 brpm	2h	1h	1	18 brpm	Aug 15 to Aug 16
4	High Breathing	55 brpm	2h	1h	<1	19 brpm	Aug 22 to Aug 24
5	Elevated Breathing	53 brpm	6h	1h	<1	18 brpm	Aug 08 to Aug 17
6	Elevated Breathing	52 brpm	5h	1h	1	19 brpm	Aug 20 to Aug 24

The P5 to P95 heart rate spread was 28 bpm (43 to 71), indicating significant cardiac variability.

Clinical Pattern Observations:

- 1. **Sustained finding:** 7h continuous Low Heart Rate on Aug 12.
- 2. **High variability:** HR spread 28 bpm (42 to 71).
- 3. **Clustered pattern:** Episodic events on 6 of 31 days (19%).



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.